DPP	DEPARTMENT POLICY AND PROCEDURE		
Title	**ASSESSING LEVEL OF CONSCIOUSNESS BY GLASGOW COMA SCALE**		
Department	**Pediatric Intensive Care Unit**		
Applies to	**All PICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-PICU-002 (v2)**		
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1.0 PURPOSE

1.1. To evaluate the neurological status.

1.2. To assess intracranial integrity

1.3. To detect a life threatening complication

1.4. To record objectively the patient's response to the environment in three major areas: eye opening, movement and verbalization.

2.0 DEFINITION/ABBREVIATION

2.1. **GCS**- Glasgow Coma Scale

2.2. **IPSG** - International Patient Safety Goals

2.3. **Glasgow Coma Scale** - is a neurological scale that aims to give a reliable, objective way of recording the conscious state of a person for initial as well as subsequent assessment. A patient is assessed against the criteria of the scale.

2.4. **Terms to explain level of consciousness**:

2.4.1. **Awake** - may sleep more than usual or be somewhat confused on first awakening, but fully oriented when aroused.

2.4.2. **Lethargic** - Drowsy but follows simple commands when stimulated.

2.4.3. **Obtunded** - arousable with stimulation; responds verbally with a word or two; follows simple commands; otherwise drowsy.

2.4.4. **Stuporous** - very hard to arouse; inconsistently may follow simple commands or speak single words or short phrases; limited spontaneous movement.

2.4.5. **Semi-comatose** - movements are purposeful when stimulated; does not follow commands or speak coherently.

2.4.6. **Comatose** - may respond with reflexive posturing when stimulated or may have no response to any stimulus.

3.0 RESPONSIBILITY

3.1 **Physicians** are responsible to assess patient and order medications.

3.2 **Head Nurse** responsible to evaluate staff nurses about their knowledge in assessing patient's level of consciousness using the Glasgow Coma Scale.

3.3 **Charge Nurses** are responsible to monitor proper assessment of GCS score.

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3.4 **Staff nurses** are responsible to carry out orders, assess GeS score and re-assess patient in a timely manner and to report any signs of deterioration or improvement.

4.0 POLICY

4.1. Critical care staff must be familiar with the Glasgow Coma Scale.

4.2. Qualified staff must know how to do neurological assessment.

4.3. Family history should be considered to assess possible genetic disorder with neurologic manifestation.

4.4. When performing a neurological assessment, the critical care nurse must focus on mental status, cranial nerve functioning and motor status.

4.5. Expected verbal and motor response must be related to the child's age.

5.0 PROCEDURE (S)

5.1. ***Preparatory phase***

5.1.1. Identify the patient by using two identifiers (Name and medical record number - first identification)

5.1.2. Ensure that the procedure and desired outcome was explained to patient's family, if present. Help to allay anxiety and get family cooperation

5.1.3. Gather material and equipment for the procedure for easy accessibility.

5.1.4. Complete the initial assessment by checking vital signs.

5.1.5. Obtain family history from parents regarding possible genetic disorder with neurologic manifestation.

5.2. ***Performance phase***

5.2.1. Assess patient's level of consciousness as follows:

5.2.1.1. Awake

5.2.1.2. Lethargic

5.2.1.3. Obtunded

5.2.1.4. Stuporous

5.2.1.5. Semi-comatose

5.2.1.6. Comatose

5.2.2. Examine body surface for the presence of injury, needle marks, petechiea, bites and ticks.

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5.2.3. Assess for pupil size and reactivity.

5.2.4. Assess for motor function: Observation of spontaneous activity, posture, and response to painful stimuli.

5.2.4.1. **Localizing Pain**- An appropriate response is to reach up and above shoulder level toward the stimulus.

5.2.4.2. **Withdrawal**- An appropriate response is to pull the extremity or body away from the stimulus.

5.2.4.3. **Decorticate posture** - adduction of the arms at the shoulder, the arms being flexed on the chest with the wrists flexed and the hands fisted, and the lower extremities being extended and abducted.

5.2.4.4. **Decerebrate posture** - characterized by rigid extension and pronation of the arms and legs.

5.2.4.5. **Flaccid**- No motor response in any extremity.

5.2.5. Examine cranial nerve reflex.

5.2.6. Check patient's best verbal response. Use Glasgow Coma Scale for documentation.

5.3. ***Follow up phase***

5.3.1. Documents the findings in flow sheet nurse's progress note and nursing care plan Document nursing assessment and intervention rendered to patient. Records any changes in neurologic status

5.3.2. Monitor any changes in neurologic status and report changes as indicated.

6.0 EQUIPMENTS

6.1. None

7.0 ATTACHMENT/FORMS

7.1. N/A

8.0 REFERENCES

8.1. MOH Guidelines for Pediatric Care

8.2. DPP-PICU-0024(V2) –Care of Patient with Endotracheal Tube– PMSH, 15/2/1439

8.3. Saudi Central Board for Accreditation of Healthcare Institution (CBAHI) PICU standards, 3rd version, January 2016

9.0 REVISION CLAUSE

9.1. Used the new logo for AL-Baha Health Cluster.

9.2. Modify the policy and procedure lay out, changed the content of the Header and footer

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